A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 May 2026

Dr Amanda Wright
East Brunswick Medical Centre

Dear Dr Wright,

Re: Ms Mary Ellen Jordan
DOB: 12/11/1973 Phone: 0402 481 745
Address: PO Box 281, BRUNSWICK EAST, VIC, 3057
Email: maryellen.jordan@iinet.com.au

Summary

- **Colonoscopy revealing a benign polyp with no evidence of dysplasia.**
- **Gastroscopy demonstrating stable focal intestinal metaplasia and healing gastric and duodenal ulcers.**
- **Negative for Helicobacter pylori.**
- **Recommendation for five-year surveillance gastroscopy and colonoscopy.**
- **No immediate action required by you.**

I had the pleasure of reviewing Maryto discuss the results of her recent gastroscopy and colonoscopy.

Regarding the colonoscopy, I did identify a polyp which I initially suspected might be dysplastic; however, the histology has confirmed this was a benign hyperplasticity polyp. The remainder of the colonic mucosa was entirely normal. Given her history of a previously dysplastic polyp - SSA, I had originally considered a three-year surveillance interval, but in light of these benign findings, I am comfortable extending her next surveillance colonoscopy to five years.

The gastroscopy was performed to monitor a known area of intestinal metaplasia and to investigate previous ulceration. I am pleased to report that the intestinal metaplasia at the base of the stomach remains stable and has not progressed since her initial scope three years ago. Furthermore, while there were small ulcers present in the duodenum (histologically normal) and evidence of a previous gastric ulcer, these areas appear to be healing well. Biopsies were negative for Helicobacter pylori. We discussed her history of frequent NSAID use for migraines and neck pain, which I suspect is the primary driver of her peptic ulcer disease.

Impression and Plan

I have advised Mary Ellen that her risk profile regarding the intestinal metaplasia remains low, as the condition is focal and she lacks high-risk factors such as a family history of gastric malignancy. I have recommended she continue her pantoprazole 40mg daily for one further month to ensure complete ulcer healing before ceasing the medication.

I have provided her with a long-term prescription for pantoprazole to keep on hand, with the strict instruction that any future use of non-steroidal anti-inflammatories must be accompanied by a PPI to mitigate the risk of recurrent ulceration. We discussed red flags that would necessitate an earlier review, including persistent

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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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epigastric pain, early satiety, melaena, or haematemesis. Otherwise, I have scheduled her for a repeat gastroscopy and colonoscopy in five years' time. Thank you for referring her back to my care.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Leila Cheikh

Investigation Results:

JORDAN, MARY ELLEN : SVH 717219, MHW 272769, SVP 629960

DOB: 12/11/1973 Age: 52 years

26 St Phillip St, Brunswick East 3057

Results from 24/04/2026 ..... Episode# 26P266224 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B010760 **CLINICAL NOTES** Gas ##Illegible## antral ulcer =? IM. Duo, D1= normal, D2= ulcer. Pt has Hx of coeliac. Sigmoid polyp=? HGD. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Gastric bx". Four mucosal fragments measuring 3-5 mm. A1. 2. Specimen labelled "D1 bx". Two mucosal fragments measuring 2-5 mm. A1. 3. Specimen labelled "D2 bx". Four mucosal fragments measuring 2-4 mm. A1. 4. Specimen labelled "Sigmoid polyp". A tan polypoid fragment measuring 10 x 8 x 1 mm. Margin marked. 3 TS. A1. Please note: Associated vegetable matter not processed. **MICROSCOPIC DESCRIPTION** 1. Sections show fragments of gastric mucosa including muscularis mucosae. There is focal intestinal metaplasia without dysplasia of complete type. Background mild chronic chemical gastropathy is noted. No ulceration or granuloma formation is present. Helicobacter pylori micro-organisms are not identified. 2. Sections show fragments of duodenal mucosa including muscularis mucosa and Brunner's gland. The villous architecture is preserved. No intra-epithelial lymphocytosis is seen. No acute inflammation, ulceration or granuloma formation is noted. No dysplasia is present. 3. Sections show fragments of duodenal mucosa including muscularis mucosa and Brunner's gland. The villous architecture is preserved. No intra-epithelial lymphocytosis is seen. No acute inflammation, ulceration or granuloma formation is noted. No dysplasia is present. 4. Sections show fragments of benign hyperplastic polyp. No dysplasia is seen. **PATHOLOGIST'S OPINION** **1. Gastric biopsy: - Focal intestinal metaplasia without dysplasia. - Background mild chronic chemical gastropathy, benign. 2. D1 biopsy: Within normal morphological limits. 3. D2 biopsy: Within normal morphological limits, no ulceration present. 4. Sigmoid polyp: Hyperplastic polyp.** Pathologist: Dr Zakariya Kanaan

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